

1. Administrative & Study Identification

Full Study Title: Korean Post Marketing Surveillance for ELREXFIO (Elranatamab) **Short Title/Acronym:** ELREXFIO PMS Korea **Posting ID:** 827806790010543763 **Protocol Number:** C1071029 **NCT Number:** NCT06581848 **Sponsor/Company Name:** Pfizer **Investigational Product:** Elranatamab (ELREXFIO) **Phase of Development:** Not Applicable (Post-Marketing Surveillance / Observational) **Trial Status:** ACTIVE_NOT_RECRUITING **Medical Monitor:** Pfizer Clinical Affairs / Pharmacovigilance

2. Study Synopsis & Rationale

2.1 Study Objective **Primary Objective:** To determine the safety and effectiveness of Elranatamab under conditions of general clinical practice, in compliance with the post-marketing surveillance (PMS) regulations of the Ministry of Food and Drug Safety (MFDS) in Korea. **Secondary Objectives:** To gather real-world data on prescription patterns, long-term tolerability, and response rates among patients receiving Elranatamab in a real-world clinical setting.

2.2 Background & Rationale To assess the safety and effectiveness of Elranatamab in real-world clinical settings for the treatment of patients with multiple myeloma in Korea. This observational study fulfills regulatory requirements mandated by the Korean MFDS to monitor recently approved therapies, ensuring continuous assessment of the drug's risk-benefit profile in the general population. During the study period within 2 years from the launch date, a whole case enrollment should be conducted according to the protocol.

3. Study Design & Methodology

3.1 Design Framework **Study Type:** Observational (Non-interventional study) **Control Type:** Single-arm (Open-label, non-comparative) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration **Target \$\$\$:** 150 participants **Number of Sites:** Multi-center (Republic of Korea) **Estimated Study Start:** 05-Jun-2025 **Estimated Primary Completion:** 31-Jan-2030

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Adult patients (≥ 18 years of age). 2. Patients who have been prescribed ELREXFIO (Elranatamab) by their physician as monotherapy for the treatment of relapsed or refractory multiple myeloma, who have received at least three prior therapies (including a proteasome inhibitor, an immunomodulatory agent, and an anti-CD38 monoclonal antibody), and have demonstrated disease progression on the last therapy. 3. Patients with evidence of a personally signed and dated informed consent/assent document indicating that the patient (or a legally acceptable representative) has been informed of all pertinent aspects of the study.

4.2 Exclusion Criteria 1. Patients with contraindications according to the locally approved label of ELREXFIO (Elranatamab). 2. Any patient (or a legally acceptable representative) who does not agree that Pfizer and companies working with Pfizer may use his/her information.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Prescribed and administered by the treating physician according to the standard of care and the locally approved label for Elranatamab. **Route of Administration:** Subcutaneous injection. **Duration of Treatment:** Variable; determined by the treating physician based on ongoing clinical benefit and normal clinical practice. **Product Information (EMA URL):** https://www.ema.europa.eu/en/documents/product-information/elrexfio-epar-product-information_en.pdf

5.2 Concomitant & Prohibited Therapy Permitted: Standard concomitant medications for multiple myeloma symptom management and co-morbidities as per standard clinical guidelines. **Prohibited:** Any therapies strictly contraindicated by the locally approved label for ELREXFIO.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening / Baseline	Day 0	Informed Consent, Medical History, prescription of Elranatamab based on standard practice
Routine Visits	Per Standard of Care	Safety monitoring, real-world effectiveness tracking, AE collection per MFDS guidelines
End of Study	Upon discontinuation or Jan 2030	Final safety and effectiveness evaluation, documentation of treatment outcome

(Note: All assessments described in this protocol are performed as part of normal clinical practice or standard practice)

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints 1. **Incidence of an adverse event (AE) / adverse drug reaction (ADR)**: The numbers and proportions of patients experiencing AE and ADR will be summarized with 95% CIs in addition to their occurrence frequencies. 2. **Incidence of a serious AE (SAE) / serious ADR (SADR)**: The numbers and proportions of patients experiencing SAE and SADR will be summarized with 95% CIs in addition to their occurrence frequencies. 3. **Incidence of an unexpected AE (UAE) / unexpected ADR (UADR)**: The numbers and proportions of patients experiencing UAE and UADR will be summarized with 95% CIs in addition to their occurrence frequencies.

7.2 Secondary Endpoints 1. **Objective response rate (ORR)** per International Myeloma Working Group (IMWG) response criteria as determined by investigator. 2. **Progression-free survival (PFS)** per IMWG response criteria as determined by investigator. 3. **Time to response (TTR)** per IMWG response criteria as determined by investigator.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via routine clinical practice and healthcare provider reporting. The safety analysis population will consist of all patients who received at least one dose of the study drug and had at least one follow-up safety evaluation. **Serious Adverse Events (SAEs)**: Reported within timelines mandated by Korean MFDS pharmacovigilance regulations. **Data Monitoring Committee (DMC)**: Pharmacovigilance and safety reviews managed internally by Pfizer and applicable regulatory health authorities.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all enrolled patients who received at least one dose of the study drug and had at least one follow-up safety evaluation in the real-world setting). **Sample Size Justification**: Target of $N=150$ is based on the whole-case enrollment requirement stipulated by the PMS protocol during the initial 2-year period post-launch. **Handling of Missing Data**: Handled via standard methodologies for observational/registry studies (e.g., available case analysis).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Pharmacoepidemiology Practices (GPP), MFDS regulations for post-marketing surveillance, and the Declaration of Helsinki.

Monitoring Plan: Observational data collection with periodic monitoring of clinical site records for completeness and regulatory compliance. **Audit Trail:** Standard electronic data capture (eCRF) with full audit capabilities.

11. Study Identifiers & Governance

Primary ID: C1071029 **Registry Number:** NCT06581848 **Posting ID:** 827806790010543763 **Sponsor/Lead Organization:** Pfizer **Study Title:** Korean Post Marketing Surveillance for ELREXFIO (Elranatamab). **Official Regulatory Title:** Korean Post Marketing Surveillance for ELREXFIO (Elranatamab)

12. Clinical Framework (Multiple Myeloma Specific)

Disease Area / Primary Condition: Multiple Myeloma (Relapsed or Refractory) **Preferred UMLS Name:** Postmarketing Surveillance **MeSH Code:** D009101 **HPO Code:** HP:0006775 **SNOMED ID:** 109989006 **Diagnosis Threshold:** Patients who have received ≥ 3 prior therapies (including a proteasome inhibitor, an immunomodulatory agent, and an anti-CD38 monoclonal antibody) and progressed on the last therapy. **Medical Methodology:** Standard real-world clinical practice according to approved local label. **Optimization Target:** Disease control, symptom management, and monitoring of post-market safety.

13. Study Procedures & Methodology

Management Pathway: Standard clinical pathway for relapsed/refractory multiple myeloma. **Education Protocol (HCP):** Training on the approved ELREXFIO label, MFDS reporting requirements, and management of known risks (e.g., Cytokine Release Syndrome). **Education Protocol (Patient):** Routine disease and treatment counseling by the HCP. **Quality Audit Mechanism:** Real-world data source verification and MFDS compliance checks.

14. Observation & Follow-Up Schedule

Total Duration: Follow-up until 31-Jan-2030 or patient discontinuation. **Onsite Visit Cadence:** Aligned entirely with real-world standard of care / routine clinic visits. **Remote Monitoring Frequency:** Routine pharmacovigilance tracking.

Checklist Review Interval: Periodic data entry and reporting to the MFDS.

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---	
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]					